



eCBHFA

Health in emergencies

Volunteer guide for empowering communities



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eCBHFA Health in emergencies

Volunteer guide

The International Federation of Red Cross and Red Crescent Societies (IFRC) is the world's largest volunteer-based humanitarian network, reaching 150 million people each year through our 191 member National Societies. Together, we act before, during and after disasters and health emergencies to meet the needs and improve the lives of vulnerable people. We do so with impartiality as to nationality, race, gender, religious beliefs, class and political opinions.

Our strength lies in our volunteer network, our community-based expertise and our independence and neutrality. We work to improve humanitarian standards, as partners in development and in response to disasters. We persuade decision-makers to act at all times in the interests of vulnerable people. The result: we enable healthy and safe communities, reduce vulnerabilities, strengthen resilience and foster a culture of peace around the world.

The IFRC is proud to work with USAID to help National Societies better prepare communities, National Societies, governments, civil society and the private sector to prevent and reduce the number of deaths in the aftermath of disaster and during disease outbreaks.

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Foreword

Communities that are affected by emergencies, such as a natural or man-made disaster, a disease outbreak or conflict are immediately forced to cope with the emergency's impact. Though the Red Cross and Red Crescent Movement trains staff and volunteers to provide support to those that are in crisis-affected communities, no one can get to the emergency faster than volunteers who are themselves in the community/ies being impacted. Time and time again we have seen Red Cross and Red Crescent volunteers providing much-needed support to their communities immediately after a crisis, even while they try to support themselves and their family through the emergency. This is the power of you as a Red Cross and Red Crescent volunteer – with the right knowledge and skills you can help yourself, your family and your neighbours in need, before anyone else can get there.

In the days and weeks after a crisis, we need people who understand the community. No two communities, or even families, are the same, and so the community understanding and ability to adapt our work is critical in this effort. To launch an effective response plan we need to know who might be most affected, where the biggest risks are, how people can be reached, and where to bring help. You and your fellow Red Cross Red Crescent volunteers know this information better than anyone else. You are key to making sure your community gets the help that it needs in the areas it is needed. You are also instrumental in delivering assistance, as first aiders and as people who understand the health of your community. This guide helps you build on your existing skills to understand the additional risks that emergencies pose and how you might help to prevent or respond to them.

As your community begins to recover, and external support begins to leave, you will continue to be custodians of your community's health. You can help with ongoing efforts to improve the health of your community, re-establish your community health initiatives from before the crisis struck, and re-assess your community risks to develop new initiatives that will prevent or mitigate the impact of a future emergency situation.

This guide will equip you with the basics for eCBHFA volunteers on health in emergencies – how volunteers are the first responders in any and every crisis, their roles in the response and how to link other actors to those most in need in the community. The module also outlines how volunteers shift from programme cycle to disaster cycle and back as the crisis evolves and the unique role of CBHFA volunteers.

Using the guides and toolkit

Health in emergencies volunteer guide

This guide is designed for community volunteers. It helps volunteers understand their unique role as true first responders in the immediate aftermath of any crisis. The guide contains key information and discussion points to work with community members in understanding their role in programming and crisis response to ensure those who are most at risk are being served appropriately.

The volunteer guide is comprised of four topics which include knowledge, tools, talking points and activities for teaching in an interactive way.

Health in emergencies toolkit

The toolkit is designed to support both the volunteer guide and the facilitator guide with interactive tools, games and illustrative material. The toolkit can be adapted and modified to local contexts, taking advantage of both local expertise and knowledge and differences in Ministry of Health approaches. Materials have been specifically designed to be printed in black and white or full colour, and with low-resource environments in mind.

Health in emergencies facilitator guide

This guide is designed for staff or trainers to conduct a training of trainers (ToT) or preparation course for staff and volunteers.

The facilitator guide allows volunteers hands-on experience and practice with all the toolkit materials so that they will be able to teach the community about their role in health following a crisis.

Health in emergencies topics

Topic	Contents
1	Programme cycle and disaster cycle
2	Emergency teams
3	eCBHFA volunteers as vital link between community and emergency teams
4	Emergency teams

Topic 1

Programme cycle and disaster cycle

Overview

This topic outlines the stages of the programme cycle, where the focus is on development of prevention and preparedness activities, and the stages of the disaster cycle and describes the volunteer's role in each stage of both cycles. If an emergency occurs, the volunteer will learn how to shift from the development stages to the disaster stages as the need arises. eCBHFA volunteers play a critical role in health emergencies. The role of the volunteer at each stage of the programme cycle and the disaster cycle is explained. Volunteers prepare for an emergency or disaster conditions through the implementation of the Red Cross Red Crescent programme cycle. Here is where volunteers assess, plan, implement, monitor and evaluate programmes or projects in their communities that focus on prevention and preparedness. The disaster cycle then details the disaster management cycle stages of any disaster or outbreak.

Learning objectives

By the end of this session, learners will be better able to:

- Identify stages of the Red Cross Red Crescent programme cycle focusing on prevention and preparedness and the role of the eCBHFA volunteer at each stage
- Identify stages of the Red Cross Red Crescent disaster cycle and the role of the eCBHFA volunteer at each stage
- Explain how an eCBHFA volunteer shifts the focus of their work from programme needs to disaster needs appropriately

Tools

- Programme cycle poster
- Disaster cycle poster

Programme cycle

The programme cycle shows the stages of a programme or project which are linked. The different phases of the project/programme cycle are:

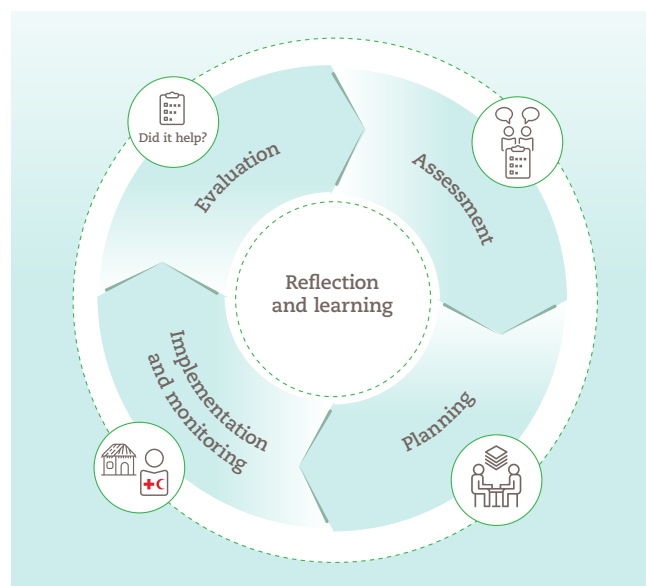
- Assessment
- Planning
- Implementation and monitoring
- Evaluation

There are different activities that typically occur in each of the four phases of the cycle. Each phase usually leads into the next, but the phases are also linked and may at times overlap. Programmes and projects have a beginning and an end, but it may be necessary to repeat certain stages when the need arises.

The type, duration and importance of activities related to each phase will vary depending on the context and the community and health needs of each unique community.

Assessment

Assessment is an important step in starting any project or programme. Assessment is the only way to understand the current capacities and vulnerabilities in a community and find out what activities or actions are needed. Assessment helps us to identify the resources, the issues or problems and the causes of these issues. We also learn about the needs, interests, capacities and limitations of the community. An assessment will highlight some of the existing issues for specific community groups.



The eCBHFA volunteer's role includes identifying the right assessment tools to use and knowing to whom to talk to find out answers. Volunteers can also prompt a community for a follow-up assessment as the community's needs change, for example when a community is struggling with an issue and needs some help in finding the right response to address it. The assessment is the foundation of any programme or project and so should accurately reflect the community's needs. Once an assessment has been conducted, this information is essential to developing a plan in the next stage.

Planning

Planning involves working with the community that was assessed to review the assessment data, analyse what was gathered and discuss the results to develop an action plan. Planning involves eCBHFA working with the community to find out the root cause of the problems that were identified. By learning **why** these problems are happening, we can ensure that the activities address the cause rather than just the symptom. Planning ensures that the activities are appropriate and possible given the resources and any barriers identified. Planning considers the needs, interests, resources, mandates and capacities of the National Society, partners and community members. During the planning phase, an action plan is developed and made ready to implement.

Implementation and monitoring

Implementation involves doing the activities that you defined in your planning. In implementation, different activities are carried out to address root causes of the problems identified during assessment. Implementation might include conducting household visits when a woman is soon expecting a baby to answer her questions and ensure that she is ready for the experience, holding a community meeting to discuss the best way to keep a water tank free from mosquito larvae in the midst of a dengue outbreak, having a cooking demonstration to show how to prepare healthy meals for a community of mothers whose children are obese, taking an older community member to the clinic, cleaning up a community or any of the thousands of activities that volunteers carry out every day. These activities build community resilience and make for healthier communities. Implementation is the “doing” of eCBHFA activities with and for the good of the community.

Monitoring is regularly collecting and reviewing information about the activities and the people they aim to help in order to track progress and make sure the activity is helping the community the way it was planned. Monitoring activities are decided during the planning phase, such as what information will be collected and when. Monitoring tells us whether or not an intervention needs to be changed or adapted to meet the community's needs.

Red Cross Red Crescent volunteers are the main implementers of any eCBHFA programme or project. They are THE key to implementation success. Without volunteers, implementation is simply not possible.

Evaluation

The evaluation phase determines if a project, programme or policy is achieving or has achieved what it set out to achieve. The aim of evaluation is to measure the relevance and achievement of objectives, as well as the efficiency, effectiveness, impact and sustainability of the project or programme. An evaluation provides information that is credible, useful, includes lessons learnt and next steps.

eCBHFA volunteers can contribute to evaluation by sharing lessons learnt, data from the community about existing activities, successes and challenges. This kind of information helps to build stronger future activities.

Reflection

1. What are the stages of the programme/project cycle?
2. What is the eCBHFA volunteer's role in each of the stages?
3. What can happen if we do not do an assessment before we plan activities?

Disaster cycle



The disaster cycle details the disaster management cycle stages of any disaster or disease outbreak. The stages include:

- Preparedness and mitigation
- Disaster or outbreak
- Response
- Recovery

Preparedness and mitigation

Preparedness is the stage where volunteers lead community members in activities that prepare them for the anticipated crisis. If the community experiences frequent disasters such as earthquakes, volunteers will regularly review with community members key strategies to keep safe during and after an earthquake. If a community receives a warning that a hurricane is coming, the Red Cross Red Crescent volunteer can tell the community members to evacuate or how to prepare to face the storm in safety.

Mitigation activities aim to reduce risks that may pose a threat. When you assessed the community, you saw existing risks that can be addressed through eCBHFA activities that may then reduce the likelihood of harm when a crisis occurs.

For example, in a community that experiences floods during every rainy season, the community can mitigate some of the risks before the rainy season even comes. They might work together to:

- plant trees along the riverbank to slow soil erosion and make flooding less likely
- move some homes and structures away from where the floods come every year
- advocate to local government to improve sewage or drainage system for streets
- conduct regular garbage clean-up campaigns to keep trash from clogging the river or drainage system when rains come

These activities remove or mitigate some of the problems that are the basis of the rains causing such damage.

Crisis event

The crisis could be a natural disaster like an earthquake or flood or a manmade disaster like conflict or war. The crisis could also be a disease outbreak that requires a response to prevent further disease or death. In the case of some diseases like measles or cholera, a single case is defined as an outbreak as the disease is highly contagious and must be responded to immediately.

Response

Response is the stage where the community and responders are reacting to the event and the problems caused by the event. For example, the earthquake is terrifying but the community is reacting to people trapped in damaged or collapsed buildings, people requiring first aid and community members looking for loved ones.

The response starts as soon as the disaster or outbreak has occurred and continues until the community begins to resume its regular pace or activities. The length of the response phase varies depending on the severity of the disaster or the number of people diagnosed with a specific communicable disease. The needs of the community change frequently throughout the response stage as people experience the different results of the event.

For example, there may have been floods that have killed many people, many people are missing and/or there is widespread damage. The community initially needs basic first aid, psychosocial support, shelter, clean water and food. Many people may still be in danger and need to be rescued. The community will also need a system to identify the dead as well as the missing. The community needs a way to reunite loved ones, like through the Restoring Family Links programme of the International Committee of the Red Cross. People need psychosocial support. Children need to attend school to build back routine and seek community with classmates and teachers in the aftermath. As routines start to return, some people may need help in re-establishing their livelihood or starting a new livelihood.

During these activities, eCBHFA volunteers provide basic health messages to keep people from becoming sick, especially if there are poor living conditions, a lack of clean water and reliable food. eCBHFA volunteers help people to recover from the shock of the disaster and prevent violence that may occur when people become overstressed. eCBHFA volunteers are best placed to assess communities' changing needs during response, while also communicating these needs to health authorities.

Recovery

Recovery is the stage where the community begins to resume regular activities again and needs less support to continue daily life. Recovery includes learning from the disaster and the response to identify what the community should do to be more resilient and less likely to be so badly impacted the next time a crisis occurs. Recovery is only effective when the community leads the way in strengthening its members and mitigating as much risk as possible for the next disaster or outbreak.

Shifting cycles when a crisis occurs

Many times, an eCBHFA volunteer will be working on a development project when a disaster occurs. While the project is important, the eCBHFA volunteer's role as a first responder takes greater importance. The eCBHFA volunteer is most likely to be the first person there when a crisis affects their community as this is where they live, work or play. When a crisis occurs, the volunteer should first and foremost provide first aid using the basic principles of first aid: assessing the situation for safety, planning how to assist, implement the first aid skill and evaluate to see if the person is responding well.

eCBHFA volunteers also provide psychological first aid as needed during and after a crisis to ensure that people's mental well-being is addressed. When emergency responders show up on the scene, the eCBHFA volunteer reports to them about who they helped, what they did, and who still requires assistance. The eCBHFA volunteer should also share the location of vulnerable people who need to be checked up on to see if they need assistance. The eCBHFA volunteer should follow up by making sure that someone has checked in with these vulnerable people and provided assistance if necessary.

As specialized responders such as ambulances or police start to arrive at the scene of the disaster or outbreak, the eCBHFA volunteer helps by providing relevant information about the community. eCBHFA volunteers should seek these teams out to inform them of:

- location of health centres before the event
- status of health centres after the event
- community members who are most vulnerable and may require extra care and attention such as disabled, pregnant, older or younger people
- community members with chronic health conditions and who consistently need medicine such as diabetic patients needing insulin or HIV+ people who need antiretroviral treatment (ARV)
- rumours, fears or beliefs that the community members may have that may negatively affect the planned response
- strategies to engage with the local community such as their preferred mode of communication – community meetings? SMS?

Once the immediate danger has passed

Once the immediate danger has passed, the eCBHFA volunteer should work with the community to re-assess the community. The volunteer and the community should meet to discuss the findings and work together on a plan to respond as necessary and slowly start to rebuild and recover. By re-assessing and revising the original action plan, the eCBHFA volunteer includes activities that address the new issues and concerns identified. By working on issues caused by the crisis, eCBHFA is helping to build a stronger community that can better prepare for future crises.

Reflection

1. What are the stages of the disaster cycle?
2. How do the stages of the disaster cycle differ from the stages of the programme/project cycle?
3. If an eCBHFA programme is in the planning stage and a crisis occurs, when and how does a volunteer shift to the disaster cycle?
4. How does the community move from the response stage to the recovery stage?
5. How do eCBHFA volunteers work on disaster cycle activities as part of their regular programme/project cycle activities?
6. Have you ever responded to a disaster in your community? What did you do? If you are eCBHFA trained has it been useful in being able to respond effectively?

Red Cross Red Crescent emergency tools used by eCBHFA volunteers

Red Cross Red Crescent emergency tools include basic health guides and tools to be used with communities. The tools can be selected based on the community's needs. eCBHFA tools can be used in every phase of both the programme cycle and the disaster cycle.

- **Epidemic Control for Volunteers (ECV)** is an interactive toolkit eCBHFA volunteers can use in the event of an outbreak or epidemic or when they are expecting an outbreak of a specific disease to occur. ECV is an extension of the eCBHFA Communicable disease prevention module.
- **Community-based surveillance (CBS)** is a toolkit, (hardware and software) that tracks specific health symptoms in a community to try to stop disease from becoming an outbreak or epidemic. While eCBHFA volunteers should monitor and report any unusual health symptoms or events in the community, CBS is a more sophisticated system for identifying threats in the community and is a tool used at the discretion of the National Society.
- **Oral Rehydration Points (ORP)** are tools where eCBHFA volunteers physically set up and maintain oral rehydration solution distribution to community members in the event of acute watery diarrhoea outbreaks.
- **Disaster Response Tools** are IFRC's response teams which may come to assist in the response after a large scale or severe disaster or outbreak. Emergency teams need Red Cross Red Crescent volunteers to inform them about the local situation and support them in their work in the community. eCBHFA volunteers are the critical link between the community and the local health authorities or responders' teams.

Reflection

1. What are the different tools used by Red Cross Red Crescent volunteers?
2. Give an example of when and how each tool might be used?
3. Which tools have you used as a Red Cross Red Crescent volunteer?

Topic 2

Cycle of health needs post-emergency

Overview

This topic discusses the cycle of health needs post-emergency and explains the role of the eCBHFA volunteer during this critical time. Red Cross Red Crescent and eCBHFA volunteers have a vital role to play in the aftermath of an emergency. eCBHFA volunteers are critical as they have first-hand knowledge about their community and its health, including knowledge of past assessments and disease prevention measures.

Based on experiences from previous emergencies, the cycle of health needs post-emergency can be somewhat forecast and planned so as to reduce the number of people needing assistance or even save lives.

Learning objectives

By the end of this session, learners will be better able to:

- Understand the ongoing impacts of a crisis situation, such as a disaster or disease outbreak, on health
- Prepare for potential health impacts following a crisis

Tools

- ❑ Needs for healthy living poster
- ❑ Emergency health action game

Needs for healthy living



Needs for healthy living poster

In communities where eCBHFA volunteers live, work and play, they are often the first responders in any crisis. Many eCBHFA volunteers will be working in the communities before any crisis occurs, meaning that they can sometimes help communities to prepare for an emergency situation and thus reduce its impact. To better understand the areas where people may need help, eCBHFA volunteers can learn about the needs that we all have through the needs for healthy living model. The six basic needs are:

- Water and sanitation
- Food
- Health
- Shelter
- Safety and security
- Livelihood

All of these basic needs are connected. Each basic need depends on the other basic needs to ensure healthy community members. When any of these basic needs are disrupted, such as during or after a crisis, it can affect people's ability to cope and thrive. All these needs must be met for people to feel safe and secure. eCBHFA volunteers are well placed to provide support to their community to address all six needs and meet the needs of community members.

eCBHFA volunteers understand the connection between health and the behaviours people practise, including hygiene, accessing and using safe water and sanitation and good food hygiene. Where assistance may be needed to meet the basic needs includes providing safe shelter, providing food and water, teaching a livelihood and giving the feeling of safety. Response to any crisis should address all of these basic needs evenly to ensure that volunteers and other responders work together to help people.

Reflection

1. What do we need to have a healthy life for ourselves and our families?
2. What are the six basic needs as outlined in the needs for healthy living model?
3. Which basic needs are important to address when responding to a crisis in your community?

Red Cross Red Crescent volunteer role during and after a crisis event

eCBHFA volunteers and health staff have an important role in all phases of a crisis. In the first phase, we work to save lives, locate and assist vulnerable people and provide first aid. Soon after, eCBHFA volunteers and staff develop and

share health promotion messages and teach about hygiene promotion to prevent potential disease outbreaks. Access to health care, safe water and sanitation, food and shelter (our basic needs for healthy living) determine how well affected people cope with the effects of the crisis.

The role of volunteers during and after a crisis is often focused on basic first aid and psychological first aid. Depending on the crisis, other support like distribution of food, tarpaulins, jerry cans, medical care and support, psychological support services (PSS), and access to safe water and sanitation commences soon after the crisis.

Vulnerable groups

We may consider some women, children, pregnant women, older people and disabled people as vulnerable people who may need additional support at any time. eCBHFA volunteers should do what they can to locate and ensure the safety of those who are vulnerable during and after a crisis event. In a crisis, volunteers know that everyone can become vulnerable for a time while those who are generally considered more vulnerable, may need even more support. Things to consider regarding vulnerability:

- Crises cause an interruption to many lives, people's homes, communities and infrastructure which may affect livelihoods as well.
- People with chronic diseases like HIV, high blood pressure, heart disease or diabetes can run out of medicine and/or cannot access their usual health services, increasing the risk of their condition getting worse.
- People may become injured as a direct result of the crisis or develop illnesses such as respiratory tract infection due to interruptions to their living conditions.
- People who are vulnerable because of their location such as remote areas or areas in conflict will need special attention from eCBHFA volunteers to check in with them regarding their needs. eCBHFA volunteers must make an extra effort to reach those who are not immediately visible in the aftermath of a crisis.
- People who are vulnerable because of their language, culture or ethnicity will also need special attention, especially if they are either a minority group, a stigmatized group or if the eCBHFA volunteer is not from their group.

Reflection

1. How can an eCBHFA volunteer help after a crisis?
2. What does vulnerability mean?
3. Who is considered vulnerable in the immediate aftermath of a crisis?
4. What is the eCBHFA volunteers' role in assisting vulnerable groups during and after a crisis?

The immediate aftermath

❏ Graphs from toolkit (First aid, Mental health, Waterborne diseases, Maternal and newborn, Vaccine-preventable diseases, NCDs, Vector-borne diseases)

Red Cross Red Crescent volunteers play an important role immediately after a crisis and in the months following the crisis. There is usually a cycle of health needs after many crises that occur across a general timeline. By understanding this cycle, eCBHFA volunteers can help communities plan ahead in the following areas:

- In the first month after a crisis event, psychological first aid and basic first aid are most often needed.
- In the first and seventh months after a big crisis, psychological support services are often needed as people process the crisis and losses immediately after the crisis and again, about seven months after the crisis has passed. Mental health and psychosocial support are most needed before and during these times.
- In the first month after the crisis and any time the season or weather change, waterborne diseases are most likely to occur. Hygiene promotion and behaviour change messages are most needed during these times.
- Mother and newborn child health are a constant area of focus. Providing support to new mothers, children and families is always needed for these vulnerable groups.
- In the first and seventh months after a big crisis, vaccine-preventable diseases occur more frequently. Conducting vaccination campaigns and messaging around booster shots during these times are most needed then.
- Noncommunicable diseases (NCDs) are a constant areas of focus. Providing support to people already diagnosed with an NCD and running healthy lifestyle activities are always needed for communities.
- In months two and three after a crisis, vector-borne diseases occur most frequently. Conducting vector elimination campaigns and behaviour change messaging are most important at these times.

Remember your community may have a different pattern or set of patterns. This guide may change depending on the extent of the crisis, your community and weather. The graphs below show timelines in months that provide a suggestion for when we sometimes expect to see an increase in particular problems or risk of disease outbreak. Although every crisis is different, there are general patterns over time that we expect to see and should be prepared to meet. These patterns allow us to predict and plan broadly for the immediate response and the next three, six and twelve months post-disaster. These patterns also help plan for the types of medical supplies and help that are potentially needed.

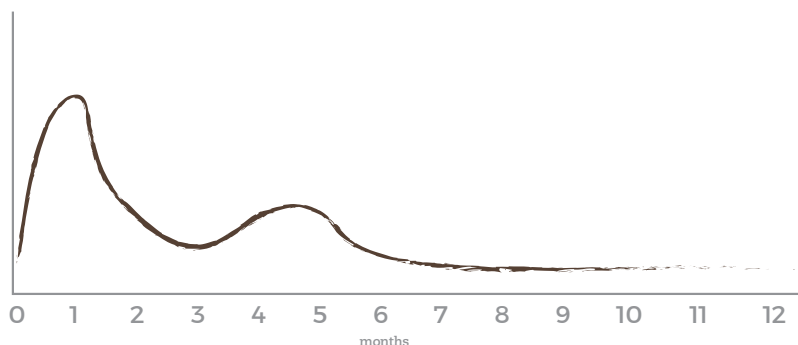
Injuries

Any crisis can result in injuries and/or deaths. Most deaths are from major injuries (like being struck by damaged or collapsing infrastructure, motor vehicles or trees) or drowning (tsunami, typhoon, floods).

Most common injuries following an earthquake, typhoon or cyclone are broken bones, head injuries, spinal injuries, damage to internal organs and internal

bleeding or lacerations. eCBHFA volunteers should ensure that their first aid skills are well-practised and teach community members the basic skills outlined in the basic first aid module of eCBHFA.

First Aid



The graph shows the incidence of trauma in the aftermath of a disaster. The peak for severe injuries is in the first one to two weeks after the disaster. We expect a secondary rise in needs once services become more accessible and available, around three to six months after the event. Some of the people seen during the second rise may be those with complications from their initial treatment and or infections, or people with a condition that requires surgery, such as an old injury, a hernia or poorly healed wound. Once normal services resume, the need for first aid goes back to normal levels.

Mental health

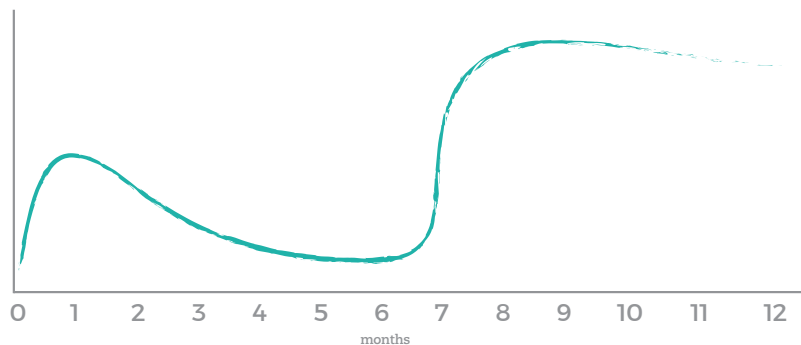
“Some of the greatest sources of vulnerability and suffering in disasters arise from the complex emotional, social, physical and spiritual effects of disasters.” - Sphere handbook p. 24.

Psychological first aid (PFA) provides emotional support to people who are affected directly or indirectly by the crisis event. People who survived the crisis may have psychological trauma or just need someone with whom to talk. Volunteers should provide PFA as soon as possible to help minimize potential psychological trauma. eCBHFA and trained volunteers have the capacity as part of their first aid training to provide PFA as a critical component of first aid. PFA can be simply recognizing that a person has been through a terrible and traumatic event and asking how they and their family are doing. PFA is about listening, comforting and treating people with dignity and respect. It is vital that PSS-trained volunteers provide psychosocial support (PSS) as needed. It is important that the volunteer provides a referral or shares any concerns about specific communities and community members with their supervisor or a PSS team member so that support can be given.

Red Cross Red Crescent staff and volunteers may themselves be affected by the impact of the crisis or through working very long days for weeks or months after the crisis event. This stress increases their risk of becoming physically sick or emotionally exhausted. Psychological support becomes a very important part of

the response for both the affected population and other volunteers and staff of Red Cross Red Crescent. Volunteers should support each other.

Mental health



Immediately after the crisis, we expect that people will need PFA to process the many emotions that may leave them feeling vulnerable, unable to cope or feel hope. PFA reduces stress and gives people support and practical help including how to locate resources, and being encouraged that all feelings are normal reactions to the event. It is also normal that some people may initially reject this offered help.

When an emergency response slows down, affected people may start to feel despair and hopelessness as things are not back to the “normal” that they knew. They may still be in temporary accommodation, schools may not be open, livelihoods have gone and health care is still not easily accessible. This often means we see a secondary rise in psychological distress after a few months. At that stage, some people may come forward for help for the first time as they were previously focused on immediate physical needs of their families or they may have initially rejected help.

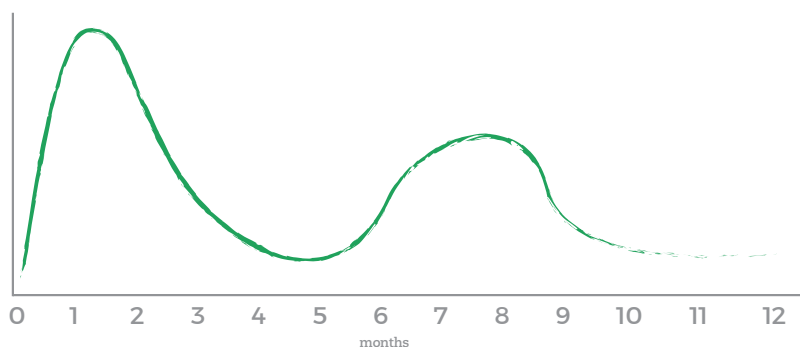
Many people require PFA more than once or may need extended PSS which can be facilitated by eCBHFA volunteers. PSS may include child friendly spaces, support groups, play activities for children, community sessions to teach about normal responses to stress and coping mechanisms, community awareness and, importantly, care for staff and volunteers. These services can be provided for as long as necessary depending on the needs, extent of the crisis and number of affected people.

Waterborne diseases

Water and sanitation are a basic need for good health, hygiene and survival. Access and quality of drinking water and sanitation are frequently and severely affected in many crises. Volunteers play a vital role in working with their community to help minimize disease outbreaks as a consequence of poor access to water and sanitation.

The initial rise in risk of waterborne disease can be due to disruption of services. It may also be that there is population movement as a result of the crisis that can lead to overcrowding and limited access to safe water and sanitation.

Waterborne diseases



eCBHFA volunteers should recognize the risk of disease outbreak soon after the crisis event and plan to help prevent disease transmission. eCBHFA volunteers should regularly ask the community to notify them of any cases of diarrhoea or deaths from diarrhoea in their families or community so that prevention and disease awareness activities can be started. People who are dehydrated or have diarrhoea which can make them severely dehydrated can become weak and are at risk of death if they do not hydrate regularly and properly.

Volunteers can help communities to prevent disease by leading by example and practising good hygiene whenever you are in the community. By demonstrating handwashing using soap and clean water at the critical times, using safe proper sanitation and proper household water storage options, eCBHFA can be the models for keeping healthy. Volunteers may also go house to house and deliver key messaging for disease prevention as necessary, specifically to vulnerable groups.

In a larger scale crisis, eCBHFA volunteers may be part of an assessment team, a first aid team or a distribution team and notice that unsafe practices are happening because of overcrowding. It is important that this information is documented and communicated to the National Society staff so a response can be planned to focus on safe practices or to help bring services to that area. Again, it is important that the volunteer listens and communicates to the health coordinator or their supervisor if they see or hear any cases of diarrhoea or any sickness in a community to help prevent a waterborne disease outbreak.

Activities that eCBHFA volunteers can do to help prevent or respond to a waterborne disease include:

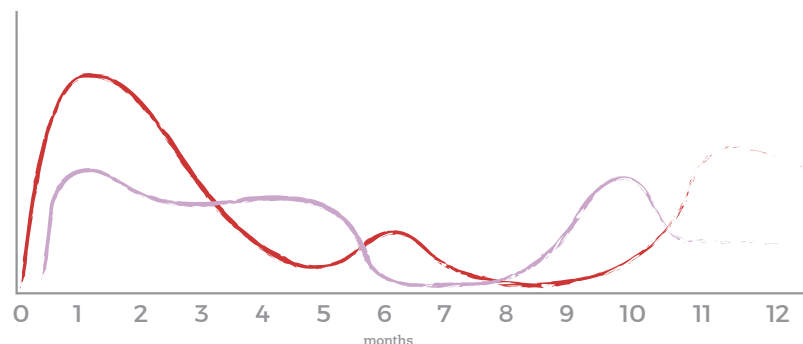
- proper handwashing activities at the critical times
- safe water messaging when distributing jerry cans or near a water source location
- safe sanitation messaging, especially in areas where people practise open air defecation
- Ministry of Health (MOH)-aligned disease prevention messaging for specific health risks like diarrhoea or acute respiratory illness
- clean up campaigns and breeding-site elimination campaigns
- training in setting up and maintaining oral rehydration points (ORP) to ensure dehydrated community members have early access to fluids

Maternal, newborn and child health

Pregnant or breastfeeding women, newborns and children under two years of age are a priority in all crises because of their vulnerability. The impact of the disaster can increase risks for mothers, babies and young children when there is:

- an interruption to food and water availability
- loss of homes
- destruction or major damage to the market place
- destruction or major damage to the health services
- loss of livelihood where husbands or men in household move away to find work
- overcrowding which can increase risk of sexual violence and rape

Maternal health issues and newborn births/health issues



The stress of the disaster may lead to pregnant women going into early labour and thus, many babies are born prematurely and require more interventions. Lack of access to hospitals or primary health care services may also impact on where the women can give birth.

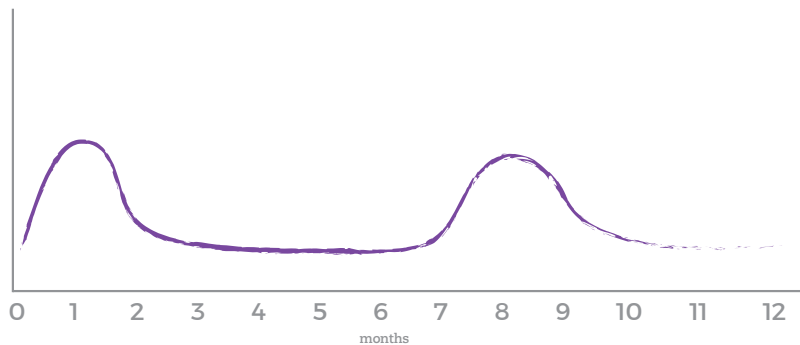
Stress from the crisis can affect a mother's breast milk supply increasing her baby's risk of acquiring a disease due to overcrowding and newborns' increased exposure to an unvaccinated population. When access to safe water is affected, volunteers should especially focus on the benefits of breastfeeding to ensure babies are getting the nutrients they need while providing PFA to women to limit stress. Volunteers can also help ensure availability and awareness of child friendly and women friendly spaces to make them more comfortable.

The cycle of needs for women and babies continues after the crisis as women continue to become pregnant, babies continue to be born, and access to services can remain interrupted for an extended period of time.

In larger scale crises, part of the emergency response includes women having access to the Minimum Initial Service Package (MISP) package, a set of tools that includes information on sexual violence, HIV, clean delivery kits, menstrual hygiene, family planning and reproductive health services. eCBHFA volunteers can provide education sessions alongside teams distributing clean delivery kits, hygiene kits and baby kits. These education sessions can teach new mothers and families about the importance of good hygiene when handling the baby, promote exclusive breastfeeding for the first six months of life, advise on disease prevention and health promotion, and when to seek help.

Vaccine-preventable diseases

Vaccine-preventable diseases



Vaccine-preventable diseases are diseases for which a vaccine exists and which is available to the whole population. There are many vaccine-preventable diseases, including measles, Rota virus, Hepatitis A and diphtheria.

When people are forced to move into overcrowded living conditions that have poor access to safe water, sanitation and hygiene, the risk of a vaccine-preventable disease outbreak is increased. It is a focus of the MoH to try and minimize the chance of an outbreak occurring when people are at higher risk. Vaccination campaigns are usually held after many crises to protect against measles or to reduce the risks of an outbreak of diseases like measles that are easily spread.

Measles

Measles is a highly contagious virus that is spread by droplets in the air, often through sneezing and coughing. Prevention of a measles outbreak relies on 95 per cent of a population being vaccinated, which can be difficult in areas with poor access to health services. Because of the crisis, affected people become more vulnerable to illness. Thus, the risk of a measles outbreak in a country with too few people previously vaccinated against measles is of great concern. While the most vulnerable people are infants under five years of age, cases can occur in children up to 15 years of age or in adults 20—40 years of age. The MoH may start a measles vaccination campaign if the number of previously vaccinated people is low.

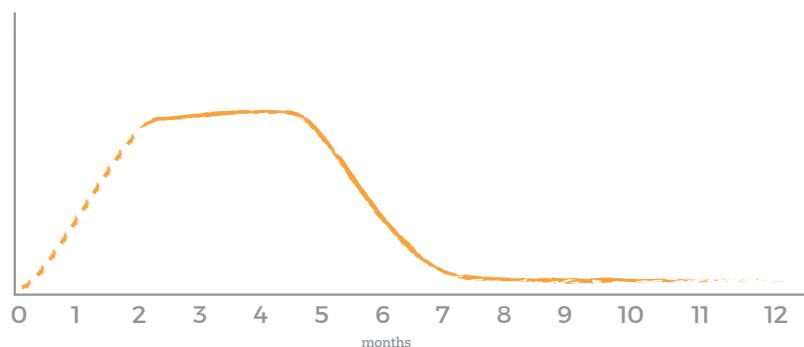
eCBHFA volunteers have strong ties with their communities and are a key contributor to vaccination campaigns. Volunteers provide awareness to the community prior to the campaign, help to get community members to the vaccination centre on the day of the campaign, and provide follow-up visits and support after the vaccination. We encourage eCBHFA volunteers who are approached by organizations to assist in a vaccination campaign to talk with their National Society supervisor to indicate any reduced availability if you do work with another organization in the short-term. Doing so does not make you less of a Red Cross Red Crescent volunteer – you are encouraged to continue volunteering with eCBHFA even if employed by another organization. And if that is not possible, eCBHFA volunteers are always welcome to come back to the Red Cross Red Crescent family when they are able.

Noncommunicable diseases

Noncommunicable diseases are diseases you cannot “catch” from someone else. There are four major categories of NCDs including diabetes, cancer, cardiovascular diseases and chronic respiratory diseases. Some 30 per cent of NCDs are caused by genetic or other unpreventable factors while 70 per cent of NCDs are preventable by making healthy lifestyle choices including no tobacco use, limited alcohol, healthy diet and physical activity.

Increasingly post-crisis, serving people who have NCDs is a major concern. Disruption to services, lack of money and stress from the effects of the disaster all affect the person with an NCD or a person whose family member has an NCD.

NCDs



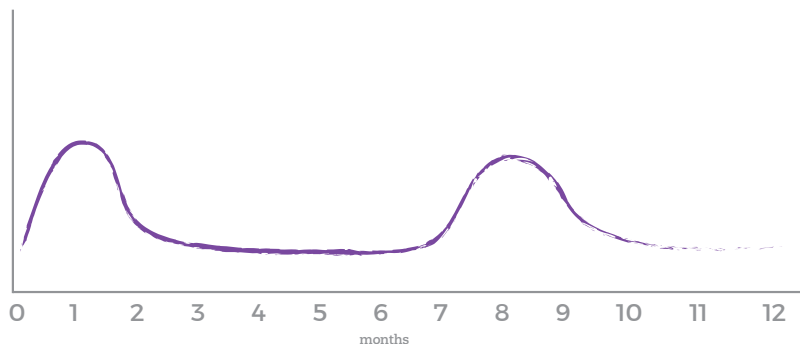
After a crisis, people are so focused on surviving, ensuring their family is together and that they have somewhere to live, they may not think about their NCD. They may or may not have had time to pack a supply of medication. This means that the rise in NCD-related issues may not be seen in the first couple of months post-crisis. When issues with people with NCDs rise, they may go on for many months until normal health services are re-established.

eCBHFA volunteers trained in NCD prevention and assessment can play a vital role in providing education sessions about maintaining a healthy lifestyle, decreasing tobacco and alcohol intake, eating a healthy diet and being physically active to minimize the risks from getting diagnosed with an NCD. Volunteers can work with health teams to access and share information with community members about availability of services, where and when services are available and how to seek help as it becomes available.

Vector-borne diseases (two to three months)

Natural disasters, especially those relating to flooding, typhoons, hurricanes, cyclones or any that attract stagnant water collection increase the risk of vector-borne disease transmission. A vector is a disease-carrying agent (mosquito, fly, tick, flea, etc.) that can cause a serious illness or death, through being bitten or some other contact. Flooding may initially wash away breeding sites but increased areas of rubbish, poor drainage and accumulation of water will increase vector breeding sites.

Vaccine-preventable diseases



Breeding sites are also affected by disasters and take time to re-establish before they become a risk to the population. When people are crowded together, there are poor water and sanitation conditions, a potentially weaker health service, and interruptions to vector control or WASH programmes, community members are at greater risk.

Volunteers can minimize risk by raising awareness about vector-borne disease transmission, vector breeding sites, poor drainage around the community and poor rubbish disposal. Vectors which are the most common include mosquitoes, rats and other animals that carry disease. Volunteers can reduce the risk of diseases caused by mosquitoes through distribution of bed nets and education on how and when to use them. Vulnerable people like pregnant and breastfeeding women, children under five years of age, older people and the disabled should be identified as a priority for mosquito net distribution.

In countries with endemic malaria or dengue fever, eCBHFA volunteers should be proactive to address vector control awareness with their community members.

You will see graphs in the toolkit that combine all the above areas where eCBHFA volunteers might need to respond following an emergency.

Wrap-up

Overall, there is a sudden rise in risks in any crisis which has important considerations for any responding teams. This content is a guide only as risks will vary for every context depending on the crisis. The graphs show a 12 month timeline that provides some indication for when to expect an increase in particular problems or risk of disease outbreak.

eCBHFA volunteers are perfectly placed to provide first aid at the beginning of the crisis but also to prepare communities before any crisis even occurs. The impact of the crisis is only one part of the response plan and eCBHFA volunteers are a critical element to engage communities before, during and after all crises.

Reflection

- Why are volunteers advised to keep their first aid skills current?
- Why is psychological first aid so important immediately after a crisis?
- Why do you think psychological first aid is often needed several months after the initial crisis has occurred?
- How can volunteers support each other with psychosocial support?
- How are PFA and PSS linked?
- Why are waterborne diseases such a high risk immediately after many crisis events?
- In looking at the graph of needs for maternal and newborn risks, the lines do not match. Why is there a greater risk for mothers at some times and at different times for newborns?
- How can a volunteer support new mothers and their babies in a crisis?
- How can eCBHFA volunteers help to prevent vaccine-preventable diseases in their community?
- What are the four NCD groups?
- How can volunteers help people with NCDs get access to the medicine and support they need after a crisis?
- What is a vector?
- How can eCBHFA volunteers help their communities to prevent vector-borne diseases?
- What are some of the eCBHFA volunteers' greatest responsibilities before a crisis? During a crisis? After the crisis has passed?

Topic 3

eCBHFA volunteers as vital link between the community and emergency teams

Overview

eCBHFA volunteers learn their critical role in improving emergency response by linking immediate community needs with local and Red Cross Red Crescent emergency response teams.

Learning objectives

By the end of this session, learners will be better able to:

- Explain the impact eCBHFA volunteers can have in an emergency response
- Explain the role of eCBHFA volunteers as a link with emergency response teams

Tools

📄 eCBHFA in action poster

Topic summary

eCBHFA trained volunteers have much to contribute when working with emergency health teams after a disaster or crisis event. Their knowledge of the community, past prevention activities, location of vulnerable people and ability to deliver health messaging is invaluable to the team and may be critical to its success.

Communities with eCBHFA trained volunteers in place pre-crisis should emerge from the emergency stronger and better equipped to address future crises as community members continually address health needs with their community health volunteers.

eCBHFA volunteers' role when working with emergency health teams

You are a Red Cross Red Crescent volunteer and you are one of the most important elements in building a stronger and healthier community! You live in your local community, whether it is a village, a neighbourhood, a town or a city. As a volunteer you work to build a stronger community, provide health services or help respond to a disease outbreak.

As an eCBHFA volunteer, you are regularly assessing for and addressing health needs of a community pre-emergency and have already identified much of the critical information about the community. This information allows eCBHFA volunteers to work with the community to address their ongoing health needs in the areas including, but not limited to:

- first aid
- psychological support services (PSS) and protection
- road safety
- malaria and dengue prevention
- nutrition
- immunizations
- assessments
- disease prevention
- water, sanitation and hygiene promotion
- Maternal and Newborn Child Health (MNCH)
- noncommunicable diseases
- tuberculosis
- HIV and sexually transmitted diseases
- diarrhoea and dehydration
- addressing stigma and discrimination

During an emergency, there are many advantages to having a Red Cross Red Crescent volunteer from the community affected because you know the community where you serve and work. This means that you know the people, the culture, the way that people interact, and you have relationships with community members and can communicate with different groups. Some of the advantages of volunteers coming from within the community are:

- you understand the community
- you know the people and understand how they think
- you know the community leaders
- you can talk to people in their language
- you know how to convince people to help themselves
- you know the local habits
- you know the local restrictions and opportunities
- you are available most of the time and you want to help others

- your community trusts you, and they will be more willing to confide their concerns, fears, beliefs, etc.

Because eCBHFA volunteers are so connected to their communities, they are a critical link between the community and teams that may come to respond to a disaster, emergency or outbreak. Because volunteers seamlessly embed into the community they serve, some disaster teams may not even be aware of the work that you do. The National Society is advised to identify eCBHFA presence in communities to emergency teams where they respond. It is always advised that volunteers make their presence known to the teams and to explain what eCBHFA is and how you can help provide a better response.

Reflection

1. During an emergency response, what advantage does an eCBHFA volunteer offer when working with emergency health teams?
2. When should you identify yourself as an eCBHFA volunteer to an emergency health team?

Interacting with active eCBHFA programming in the affected community

Sometimes a disaster, conflict or outbreak occurs which requires outside help. If an emergency health team comes into a community, it is helpful to talk with eCBHFA volunteers who already know the community's resources and needs. This information helps first responders to work out who the neediest community members are that were affected by the crisis. A volunteer can also help by providing information about rumours or any possible disease symptoms they may have seen.

Red Cross Red Crescent volunteers should talk with first responders to provide information about what is currently happening. This allows first responders to best assess and address the current situation.

Red Cross Red Crescent volunteers should continue to work with the Ministry of Health (MoH) and local emergency authorities as well as other relevant health teams who might be involved. Often emergency health teams may not be from the country and may struggle with the language, culture and ways of effectively working in-country. Volunteers can assist these teams by helping with both assessing the immediate health impact of the disaster AND explaining the health situation and capacity of the community before the emergency. Volunteers can inform the emergency health team about the affected population as a whole, the existence and location of marginalized and vulnerable groups in the community, and the location and condition of community members that rely on continuous medication and care.

Responding to immediate health needs

Together eCBHFA volunteers and other responding teams can use information about the community before and after the emergency to assess and respond to the immediate health needs of the community post-emergency. For example, if pre-emergency volunteers have been addressing HIV and sexually transmitted infections, diabetes or noncommunicable diseases in the community, they should inform emergency health teams. In that way the emergency health teams can continue ongoing care and medications for those patients in addition to immediate health needs as part of their plan of action. Additionally, volunteers should relay other health-related information. For example, if the local hospital has a bed capacity of 50 beds, but the hospital is damaged or missing some critical equipment like a generator or sterilizing equipment, the volunteer can relay this information to the emergency health teams to help outline the local needs and capacities of health facilities.

Identifying critical needs

The emergency health teams who coordinate with other teams and the MoH will use this information to respond more quickly and efficiently. This coordination allows teams involved in a response to identify critical needs and which people and teams will address each of the identified needs. This collaboration also prevents overlap and gaps in relief activities by the responders involved in addressing the affected community's needs.

The community health volunteers and the National Society team provide key information and updates on the community situation to the emergency health team and vice versa. As an eCBHFA volunteer, you are integral to delivering critical messaging to the community and ensuring that there is continued implementation of health services and messaging to the community members who need immediate and continuous care or medications like blood pressure tablets or anti-retrovirals (ARVs) for HIV.

Special training

Depending on the crisis, your National Society staff or other emergency health people may provide some training sessions to eCBHFA volunteers to address specific problems that are beyond what you typically may do in your community work. For example, if there is a measles outbreak which requires an emergency immunization campaign, volunteers would be provided training such as the Epidemic Control for Volunteers materials and other resources. Volunteers may then be requested to help support and implement the vaccination campaign with organizations such as UNICEF, Médecins Sans Frontières and the MoH.

It is normal for regular programmes to stop during the aftermath of the emergency as volunteers become vital to the emergency response. Additionally, the community may have suffered a direct impact from the crisis and therefore programmes are unable to be continued for the time being. Experienced volunteers then turn their focus to being part of a broader emergency response team.

Working elsewhere or with other organizations

In many instances, Red Cross Red Crescent volunteers are asked to work somewhere different than their home community after a large disaster or outbreak. If this occurs your National Society will provide support to help you succeed. Sometimes eCBHFA volunteers are hired temporarily by other organizations for emergency responses or vaccination campaigns. Red Cross Red Crescent is happy for eCBHFA volunteers to carry out this work as it helps people in need. If this occurs, make sure to inform your supervisor at the National Society. They will ensure that you have the support you need during and after the work to be able to carry on with your eCBHFA duties upon your return. Red Cross Red Crescent is part of who we are as volunteers and eCBHFA volunteers will always have a home in the Movement.

Reporting on changing needs

Post emergency, health situations are continually monitored since the scenario can change so quickly. As a result, the health needs of the affected population will change. eCBHFA volunteers should make sure that they monitor and report on the community's changing health needs as the aftermath of the crisis changes and poses new challenges. The National Society may monitor and report on the political, financial and logistical situations as well as monitor resources available and gaps in necessary support to the affected communities.

It is not appropriate or possible for emergency health teams to monitor the full post-crisis situation on their own. While they may be experts in the technical area of treatment and care, they are not experts on the context of the community or the country. Without knowing the many facets that comprise the context of the situation, there are too many gaps for emergency health teams to craft a comprehensive, quality response that meets the greatest number of community members' health needs. This is why eCBHFA volunteers are such a critical part of a good emergency response. Volunteers are simply the best way to ensure community members have what they need to work towards rebuilding their resilience, health and community.

Evaluating the response

With the eCBHFA and community health team, the emergency health team supports the evaluation of the health response for both effectiveness and responsiveness. This occurs when eCBHFA or community health teams prepare to shift into the recovery phase. The community health volunteers and the National Society continue to address the needs of the people, re-assessing as necessary to ensure that their work addresses the community's most critical needs.

Reflection

1. Name three ways that the eCBHFA volunteer is a critical part of the response?
2. What is the one unique aspect that emergency health teams bring to the situation post-crisis? What are the many unique aspects that eCBHFA volun-

teers bring to the situation pre and post-crisis?

3. If an eCBHFA volunteer was hired to work for three months by UNICEF, what is their first step upon accepting the offer?
4. How do eCBHFA volunteers make their presence and expertise known to responding emergency health teams?

Topic 4

Emergency teams

Overview

This topic outlines the importance of communication, cooperation and team-work during a health emergency. The role of the eCBHFA volunteer is discussed when working with national and international colleagues as well as working alongside other emergency teams.

Learning objectives

By the end of this session, learners will be better able to:

- Identify initial priorities for volunteers according to the emergency
- Identify the strengths and focus of the National Society in a health response
- Gain an understanding of what the government and other organizations do in an emergency

Tools

- Emergency response mechanism poster

Role of the National Red Cross Red Crescent Society

The National Red Cross Red Crescent Society is often well placed because of its existing programme work to provide prompt and essential response straight after an emergency. Staff and volunteers are often already well known and respected at the community level. Frequently it happens that the National Society and its network of volunteers is the first responder during an emergency,

providing lifesaving assistance, shelter, water, food and basic healthcare. In addition, the initial information that they can provide to others who are drafted in to give assistance becomes vital to any further assessment planning.

Communication and coordination

Communication and coordination are vital elements of any team responding in an emergency. Communication and coordination are key elements for the eCBHFA team as well as for the National Society. But it is vital that the National Society and its volunteers form part of the overall response effort, which may include other national and international actors, and this is where communication and coordination, if effective, can ensure the most efficient response possible. Cooperation, good communication and a knowledge of how the overall response is organized can help eCBHFA volunteers to make a vital contribution to health during the response efforts.

In a major emergency many international teams arrive to provide additional assistance that is beyond the capacity of the National Society. These teams always work side by side, with the MoH being in overall charge of the response, sometimes with the support of the World Health Organization (WHO).

The overall coordination mechanism needs to ensure a clear understanding of what the National Red Cross Red Crescent Society and its staff and volunteers can do. Answers to the following questions help this understanding:

- What is needed in the immediate response? For example, it is necessary to supply first aid and distribute tarpaulins and jerry cans.
- Who can be mobilized? For example, where are the available volunteers and how many will be needed?
- What technical expertise is needed? For example, does a national team need to undertake search and rescue?
- What are strategies for looking after teams in the field? For example, what is the need for equipment and psychological support services (PSS).

Working as part of the overall response team is about identifying what strengths everyone brings, who has leadership skills, who can best ensure information is communicated and documented clearly, who best manages security of the team, how best to keep an eye on one another in the field and how best to work as an effective team.

Roles of Red Cross Red Crescent health teams

The immediate effect of the crisis means that all volunteers become part of the local response. Generally, existing programmes are halted until things are beginning to get back to normal. All volunteers are vital to the response. eCBHFA volunteers have knowledge about existing health problems, vulnerable populations,

hygiene practices and behavioural practices and can give useful information to incoming teams in these areas.

The National Society health team often has existing relationships with the MoH and other agencies. At first, the role of the National Society teams may be to provide basic first aid, retrieve bodies, provide some shelter support with tarpaulins and/or tents, provide distribution of items to promote safe collection of water and/or food distribution and to undertake initial assessments. The health teams are responsible not just for treating people with basic first aid but also as soon as possible they need to prepare to help prevent a disease outbreak, common after a disaster. The MoH might organize a mass vaccination campaign, and since Red Cross Red Crescent volunteers are globally recognized for their access and work at the community level, volunteers may be called on for their social mobilization skills and ability to deliver key messages.

Key messaging

If there is a health department in the National Society then Red Cross Red Crescent volunteers should be directed by the health coordinator. This person and his/her team start looking at the priorities for the health response. This may mean rapidly scheduling brief trainings to the volunteer team over a few hours, day or days to ensure the team is well prepared to deliver key disease prevention and hygiene messages. Key messages that can be taken from eCBHFA programme work will provide families with information promoting basic hygiene, safe water, safe sanitation and disease prevention measures. Volunteers know from past experience that when people lose their homes and move into overcrowded situations practising good hygiene can be difficult.

Recording data

As soon as possible the health team should develop data recording measures, such as the use of mobile phones to record data, to ensure Red Cross Red Crescent data can be looked at, analysed and shared with the MoH. This sharing of information ensures that the response is targeted more accurately.

Reflection

1. What is the primary focus of National Society teams in an emergency?
2. What is the primary role of eCBHFA volunteers within the health team?

National and international response teams

The information below is provided so that eCBHFA volunteers have some understanding about the various teams that may be deployed in an emergency and the role of each.

During a response, eCBHFA volunteers should be aware of the following elements:

- Red Cross Red Crescent teams/global tools
- IFRC Disaster Response tools
- Overall coordination
- Early warning and response system (EWARS)

Red Cross Red Crescent teams/global tools

The Red Cross Red Crescent has the capacity to provide emergency health assistance in both a first line and prevention approach. This response should be in alignment with the host government's plans for response. If no international support is needed during an emergency the National Society will determine how they can best respond. The National Society may request some technical support from within the Red Cross Red Crescent Movement. If the government declares a national emergency and asks for international support then the National Society may request international help on a broader scale.

IFRC Disaster Response tools

IFRC Disaster Response tools are ways that Red Cross Red Crescent teams come to assist in the response after a disaster or during an outbreak. Response teams work alongside the National Society and need Red Cross Red Crescent volunteers to inform them about the local situation and support them in their work in the community. There are five response teams, noting that not every emergency will need all these different response tools. They include:

1. National Response team (NRT)
 - Red Cross Red Crescent staff or volunteers from within the country who have undergone standardized training that provides them with additional technical expertise for assessment of damages and needs and support in relief distribution to affected populations
 - Team may be the only additional support required in smaller scale emergencies
 - Some National Societies have community and/or branch disaster teams which are managed by the National Society. Check with your National Society if they also have these teams and how you can be involved.
2. Regional Response or Regional Intervention team (RRT or RIT)
 - Red Cross Red Crescent staff or volunteers from within the region who have undergone standardized training to bring assistance to a National Society in a neighbouring country within their region. Their role is to actively help the regional capacity in disaster management. They may sometimes be deployed internationally.
 - They are made up of cross sectoral expertise such as health, logistics, relief and water and sanitation.
 - Often familiar with local language and culture.
 - Team may be the only additional support required in smaller scale emergencies
 - Some team members will have training to provide technical expertise in assessment and determining the international response in large scale disaster operations
 - Helps develop the plan of action (PoA) which is necessary for funding the

operation for as long as needed

3. Head of Emergency Operations (HeOps)

- A highly experienced Red Cross Red Crescent delegate who has undergone specialized training to take a lead role in a major emergency response
- S/he strengthens the strategic on -ground leadership capacity and planning for the operation and helps improve coordination amongst all responders
- May be deployed in large scale emergencies for three months to help provide a strategic response
- Works alongside National Society head of the emergency response

4. Public Health in Emergencies (PHiE)

- Emergency health delegate that may deploy as an individual or in a bigger emergency as part of the health Emergency Response Unit (ERU – see below)
- Community-based role to support disease outbreak or to help develop preventative strategies and plan public health response that includes capacity-building and training of volunteers, if required
- Can provide expertise and additional trainings in epidemics including disease surveillance, data collection, case management, support wider ERU response and implementation of ORPs at community level

5. Emergency Response Unit (ERU) – people and materials

- Trained teams of technical experts who are deployed in larger scale operations. ERUs are designed to be self-sufficient for one month but can operate for up to four months
- Designed to provide specific support services when local facilities are either destroyed or the impact requires additional needs
- Multiple ERUs available from different countries from emergency field hospital to mobile clinics, safe water and mass sanitation (WASH and MSM), logistics, IT and telecommunications, relief and base camp
- Deployed through the IFRC in Geneva
- Usually for up to four months but depends on the emergency
- Comes with international staff but utilizes additional local staff and volunteers during the operation

There may also be other sectoral response tools like shelter, cash, livelihoods, etc., which may also respond.

The place of Red Cross Red Crescent in the whole response

The following information is provided to help volunteers gain a basic understanding of the Movement as a whole during a response and how Red Cross Red Crescent fits into the response in terms of our responsibilities and accountabilities as a Movement.

Overall coordination

The Ministry of Health (MoH) of the affected country takes the overall responsibility and lead in the health response. It is supported in bigger emergencies or disease outbreaks by the World Health Organization (WHO) who may assist in providing technical support and expertise if necessary.

In large emergencies a coordination mechanism is set up by the MoH and WHO so teams can communicate and coordinate to ensure the best response and to minimize duplication of effort. For example, it is necessary to coordinate team locations so that responders spread across the emergency rather than all working in the same area. The Red Cross Red Crescent is often very well-respected at this level so it is vital that they attend these coordination meetings. It is a platform to share information, ask for further information and see who is doing what.

The 4Ws form (who, what, when, and where) is set up at the coordination level and all responding teams are expected to make sure they are represented on this form. For Red Cross Red Crescent this may look like:

- 40 bed field hospital with surgical and obstetric capacity for four months at location X
- three mobile clinics from Partner National Society for four months in locations X, Y and Z
- community health messaging to prevent diarrhoea outbreak in 20 communities by host National Society volunteers in locations X, Y and Z daily for one month
- community hygiene promotion and water distribution via jerry cans in 20 communities by host National Society volunteers in locations X, Y and Z daily for one month

Early warning and response system (EWARS)

The MoH is responsible for the Early warning and response system (EWARS) or public health surveillance in their country. The purpose is to collect information on diseases occurring, analyse and share the information, and develop appropriate reports and response information. The strength or weakness of EWARS may be as a result of the current emergency. The system may under-perform because of disruptions or it may have lacked strength and expertise prior to the emergency. WHO provides the necessary support to this system. Red Cross Red Crescent health teams are responsible to feed any health information to the public health team to ensure their findings are represented in the surveillance response and reports.

When there are major disruptions to people's lives and infrastructure as a result of a disaster, we know that the likelihood of disease outbreak rises dramatically. Displacement of people and overcrowding can increase the risk of transmission of communicable disease and potentially deaths. Rapid detection, reporting and response are vital and a key priority during the emergency response for teams.

Reflection

1. Who has the overall responsibility and lead in the health response of an emergency?
2. What are IFRC's Response tools? What is the purpose of each tool?
3. Who is responsible for the early warning and response system (EWARS) or public health surveillance in your country?

Local emergency teams

During an emergency, multiple national and local emergency teams may participate in response operations. These all form part of the overall team and should be coordinated by the MoH.

Police and fire

Globally, police and fire officers play an important role in emergency operations including search and rescue, evacuations, dead body removal, and, overall, maintaining the safety of the public. In past emergencies, Red Cross Red Crescent has found that police and fire officers work in a collaborative and cooperative manner.

Ambulance

Red Cross Red Crescent have an ambulance service in many countries. In an emergency this ambulance service provides first aid and emergency medical care as part of the immediate response. A volunteer may be part of the ambulance team as a skilled first aider. The ambulance team may also provide mobile clinics daily for up to a few months post-crisis.

Search and rescue

Red Cross Red Crescent volunteers can participate in search and rescue training to then be part of the squad that deploys within a country or across a region to assist in the immediate aftermath of the emergency. These volunteers are “frontliners” who are trained in risks, hazards, high intensity rescues, dead body retrieval and first aid including basic life-saving measures.

Other agencies

Very often the host National Society will already be aware of other local teams working in the country. The bigger the emergency the more local and international teams will appear. As already mentioned, the MoH are the leaders in the health response. There may be other national or international teams working in the country on long-term development programmes. Like Red Cross Red Crescent these teams may also engage their headquarters for more support. There may be existing arrangements between the host National Society and other agencies such as UNICEF.

Reflection

1. What national or local emergency teams might you encounter during an emergency in your role as an eCBHFA volunteer?
2. What roles might an eCBHFA volunteer play when working with other national or local teams?

The fundamental principles of the international Red Cross and Red Crescent movement

Humanity The international Red Cross and Red Crescent movement, born of a desire to bring assistance without discrimination to the wounded on the battlefield, endeavours, in its international and national capacity, to prevent and alleviate human suffering wherever it may be found. Its purpose is to protect life and health and to ensure respect for the human being. It promotes mutual understanding, friendship, cooperation and lasting peace amongst all peoples.

Impartiality It makes no discrimination as to nationality, race, religious beliefs, class or political opinions. It endeavours to relieve the suffering of individuals, being guided solely by their needs, and to give priority to the most urgent cases of distress.

Neutrality In order to enjoy the confidence of all, the Movement may not take sides in hostilities or engage at any time in controversies of a political, racial, religious or ideological nature.

Independence The movement is independent. The National Societies, while auxiliaries in the humanitarian services of their governments and subject to the laws of their respective countries, must always maintain their autonomy so that they may be able at all times to act in accordance with the principles of the Movement.

Voluntary service It is a voluntary relief movement not prompted in any manner by desire for gain.

Unity There can be only one Red Cross or Red Crescent Society in any one country. It must be open to all. It must carry on its humanitarian work throughout its territory.

Universality The international Red Cross and Red Crescent movement, in which all societies have equal status and share equal responsibilities and duties in helping each other, is worldwide.

For more information on this IFRC publication, please contact:

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